

Patient Grievance and Complaint Management

Policy Number	POL-CMP-082	Owner / Department	Patient Experience / Risk Management
Effective Date	2023-06-15	Version	3.0
Last Revised	2026-04-20	Next Review	2028-04-20
Approved By	Patient Experience Committee	Applies To	All staff who receive patient or representative concerns; coordinated by Patient Experience

1. Purpose

This policy establishes a prompt, fair process for resolving patient complaints and grievances consistent with the CMS Condition of Participation on patient rights (42 CFR 482.13). It distinguishes a complaint from a grievance and sets response timeframes and written-notice requirements.

2. Definitions — Complaint vs. Grievance

Type	What it is	Target timeframe
Complaint	A concern resolved promptly by staff present, about care or service that can be addressed at the point of contact	Resolve during the encounter; no written notice required
Grievance	A formal written or verbal concern about care, safety, Medicare rights, or abuse that is not resolved at once, or any written/emailed concern	Written notice of resolution; ~7-day average per CMS

A concern becomes a grievance when it cannot be resolved at the time by staff present, is postponed for later follow-up, is submitted in writing (including email), or alleges abuse, neglect, or a patient-safety or Medicare rights issue.

3. Process

1. Acknowledge the concern and attempt immediate resolution; de-escalate and address what can be fixed now.
2. If it meets the grievance definition, enter it in the grievance log and notify Patient Experience the same day.
3. Investigate: gather facts from the care team and the record; involve the manager and, as needed, Risk Management.
4. Determine resolution and document the steps taken and the outcome.
5. Send the patient or representative written notice of the decision.

4. Written Notice and Timeframes

- The grievance process targets a **7-day average** resolution; most grievances receive written notice within 7 calendar days.

- If resolution will take longer, send the patient an interim update stating that review is ongoing and a follow-up date.
- The written notice must name a hospital contact person, the steps taken to investigate, the results of the process, and the date of completion.

IMPORTANT — Any grievance alleging abuse, neglect, or an event that caused patient harm must be reported to Risk Management and, where applicable, filed as an event report per POL-RM-013 at the same time the grievance is logged — do not wait for the grievance process to conclude.

5. Severity & Reporting

A grievance is classified on the facility-wide scale in POL-RM-013 based on any underlying event. A grievance alleging serious harm or a sentinel event is SEV-1 (notify the Administrator-on-Call and Risk Management within 1 hour). A grievance alleging a major care issue without current harm is SEV-2 (notify Patient Experience and the manager within 4 hours). A routine service grievance is SEV-3 (report within 24 hours).

6. References

- CMS Conditions of Participation, 42 CFR 482.13 (patient's rights; grievance process and timeframes).
- CMS State Operations Manual, Appendix A (interpretive guidance on grievances).
- Related: POL-CMP-083 (Patient Rights), POL-RM-013 (severity & reporting).